جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نرجس سليمان فهيم شحات
المحافظة : اسيوط **الحي / المدينة :** منفلوط **العنوان :** //
رقم البطاقة : 28503092501423 **رقم التليفون :** 01206191722 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 09-03-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نرجس سليمان فهيم شحات
National ID: 28503092501423

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced CT of chest, abdomen and pelvis.**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



٢٨٥٠٣٠٩٢٥٠١٤٢٣ ٢٠٢٥/١٠

بدون عمل

أنثى مسيحية متزوجة

عماد زارع يسى مسعد

البطاقة سارية حتى ٢٠٣٢/١٠/٢٤

١



نرجس

سليمان فهيم شحات

قرية العزية

مركز منفلوط-اسيوط

٢٨٥ ٠٣ ٠٩٢٥ ١٤ ٢٣

LL1196933

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 11/1/2026 G

Name	نرجس سليمان نعيم	MRN	٣٦٤١١٠٢٥
		ID	٢٨٠٠٣٠٩٢٠١٤٢
Age	41	Phone	01281348677
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	New case
CO morbidity	n. Nil		

Clinical Data

BX: (+) IDC GII with DCIS

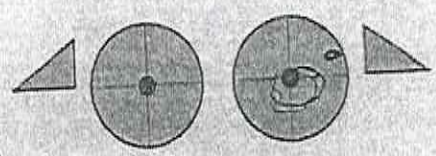
ER:	PR:	HER 2:	Ki67:
8/8	5/8	2+ 50% not amplified	35%

Sonomammography:

(+) Breast diffuse skin Thickening with edematous changes

Mets Workup: U o Q (2.5 x 3) at 11 o'clock and another one at 10 o'clock mass 4x2cm (BIRADS 4)

TNM o 4B



Recommendations: - ? well defined oval shaped mass at 6 o'clock about 6x4 mm (BIRADS 3)
- multiple enlarged axillary LNs with preserved fatty hilum

CT → Nil

Consider core biopsy

MDT OUTCOME & PLAN

Neoadjuvant DCX4 → Restaging X4 → MRI breast + Mets workup → Surgical assessment → Radiotherapy

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[DOCTYPE NOT_FORM]

Name: نرجس سليمان فهيم شحات	
MR. : 7262	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 41-year-old individual with no reported co-morbidities. The clinical history notes a left breast invasive ductal carcinoma, classified as CT4B, without metastasis (NO MO), grade IDC2, with estrogen and progesterone receptor scores of 8/8 each and HER2 in situ hybridization not amplified.

Biopsy confirms an invasive ductal carcinoma, grade IDC2, estrogen receptor 8/8, progesterone receptor 8/8, and HER2 negative (not amplified). Follow-up sonomammography demonstrates regression of the breast mass measuring 21 × 17 × 12 mm. Staging imaging with CT of the chest, abdomen, and pelvis is negative, and bone scan shows no metastatic disease.

The referring team recommends surgical consultation for surgery, specifically left modified radical mastectomy (MRM) with attention to skin edema and the use of good flaps. The MDT concluded that the patient has completed neoadjuvant chemotherapy with an initial CT4B N0M0 response, achieving a partial response and is now planned for left MRM.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

Patient Name	: الأستاذه / نرجس سليمان فهيم	Request No.	: 13002746
Patient No.	: 28503092501423	Request Date	: 10/11/2025
Path No.	: 3454/25	Verification Date	: 27/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast UOQ mass.

Nature of specimen
Tru cut needle biopsy (TCNB) , collected on 10/11/2025. at 12:55 p.m, cold ishmeia time 5 minutes .

Gross examination
Multiple grayish white tissue cores ranging in size from 1 cm up to 1.5 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS:Intermediate grade (solid &cribriform sutypes with comedo type necrosis
* Angioinvasion: Absent
*Perineural invasion: Absent
*Microcalcification : Positive
* IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
* PR: Stromng nuclear reaction in 10 % tumor cells (Allred score 5/8)
* HER2: Moderate complete membrane reaction in >10 % of tumor cells (score 2+)
*SISH :Not amplified
Average Her2 copy No :2.05
*HER2 /CEP17 ratio :1.7

Verified by :
Dr.Heba Hasan

CAP
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No (4) - First Floor Building No 987 Nile Corniche Misr Old Cairo

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / نرجس سليمان فهيم	Request No.	: 13002746
Patient No.	: 28503092501423	Request Date	: 10/11/2025
Path No.	: 3454/25	Verification Date	: 27/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

* Ki 67: 35 %

Diagnosis
Left breast UOQ mass, Tru cut needle biopsy (TCNB):
Invasive duct carcinoma, Grade 2 with intermdiate grade DCIS(solid &cribriform sutypes with comedo type necrosis)
IPT:
* ER: Positive (score 8/8).
* PR: Positive (score 5/8).
* HER2: Equivocal (score 2+).
* SISH : Not amplified
* Ki67: 35%
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY
NATIONAL

Patient Name	: الأستاذه / ترجس سليمان فهيم	Request No.	: 13002746
Patient No.	: 28503092501423	Request Date	: 10/11/2025
Path No.	: 3454/25	Verification Date	: 27/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result). Presidential Initiative for Women's Health- Pathology Lab Templates

• Group V (Negative): HER2/CEP17 ratio is less than 2

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Hebba El-Shafeey

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Women's Health Outreach Program

Name: Narges Soliman Fahim.
Date: 6/11/2025

Full field Bilateral Digital Breasts ultrasonography

Left breast

- Left breast showing diffuse inflammatory changes evidenced by diffuse skin thickening with edematous changes and echogenic fat planes , the upper outer quadrant showing irregular hypoechoic lesion measuring about 2.5X3 cm seen at 11O'clock and another one seen at 1 O'clock measuring about 4x2 cm (BIRADS 4).
- A well defined oval shaped solid lesion seen at 6O'clock measuring about 6X4 mm (BIRADS 3).
- No evidence of dilated ducts.
- Multiple enlarged axillary LNs with preserved fatty hilum.

Right breast

- Right breast showing a well defined hypo echoic solid lesion measuring about 6X4 mm seen at 2 O'clock (BIRADS3).
- Otherwise, Normal breast parenchyma with no other masses nor cysts nor focal lesions detected .
- Normal skin thickness.
- No evidence of dilated ducts.
- Multiple enlarged axillary LNs with preserved fatty hilum.

Conclusion:

-left breast two suspicious lesions as described above (BIRADS4) with diffuse inflammatory changes for further assessment.

Bilateral benign looking lesions as described likely fibroadenomas (BIRADS 3).

Best regards,
Dr. Mai Mohamed ,MSC

Sonography
[[DOCTYPE:SONOGRAPHY]]

Ain Shams University Hospitals
Radiology department

مستشفيات جامعة عين شمس
قسم الأشعة التشخيصية

Patient Name	نرجس سليمان فهيم شحات	PID	1021052510 4294	D/T	26-10-2025

SONOMAMMOGRAPHY OF BOTH BREASTS

Procedure: Digital mammography and real time ultrasonography of both breasts were performed.

Findings:

- Right breasts display normal skin thickness with normal appearance of the nipples and subareolar regions.
- Right well circumscribed oval hypoechoic soft tissue lesion at 2 o'clock 1CMFN measuring 5.8x 5.4 mm and another one on left side at 6 oclock measuring 8.5 x 4.4 mm
- Normal appearance of the subcutaneous and retromammary fat on right side .
- Left breast displays increase skin thickness reaching 6 mm with subcutaneous tissue edema.
- Left two ill defined speculated hypoechoic soft tissue lesions with posterior shadowing and internal calcifications at 11 o'clock 2 CMFN and the anther one at 2 oclock , 3 cm from nipple measuring approximately 4.3 x 2.2cm
- The breast parenchyma is heterogeneously dense (ACR breast density c). Left breast on mammogram display diffuse malignant looking microcalcifications
- No suspicious solid or cystic masses were detected in the right breast
- No suspicious axillary lymphadenopathy on right breast with reactionary looking lymphnodes on left breast with cortical thickness reaching 3 mm
- No suspicious bilateral intermammary or supraclvicular lymphnodes

Impression:

- **LT BREAST INFLAMMATORY CHANGES WITH MICROCALCIFICATIONS ON MAMO AND TWO SUSPICIOUS LESION, AS DESCRIBED ABOVE, FOR PATHOLOGICAL VERIFICATION (BIRADS 4), +CONTRAST ENHACED MAMMOGRAPHY TO EXCLUDE OTHER LESIONS**
- **TWO BENIGN LOOKING LESIONS SEEN ON RIGHT AND LEFT BREAST AS DESCRIBED , LIKLEY FIBROADENOMA (BIRADS 3)**
- **LEFT BREAST REACTIONARY LOOKING LYMHNODES**

R/Ayat Salem
Nada Khaled,Msc

Email : feedbackradiology@gmail.com

مستشفيات جامعة عين شمس - شارع رمسيس -
العباسية القاهرة

Sonography
[[DOCTYPE:SONOGRAPHY]]